



/ Courses (<https://entrada.med.ubc.ca/courses>)
/ Family Practice Postgraduate Program
(<https://entrada.med.ubc.ca/community/familypracticepostgr>)
/ Program Resources
(https://entrada.med.ubc.ca/community/familypracticepostgr:program_resources)
/ Assessment
(https://entrada.med.ubc.ca/community/familypracticepostgr:program_resources/assessment)
/ Assessment Instruments
(https://entrada.med.ubc.ca/community/familypracticepostgr:program_resources/assessment?section=view-folder&id=8459)
/ In-Training Assessment Repor ...

In-Training Assessment Reports (ITARs)

In-Training Assessment Reports (ITARs)

- In-Training Assessment Reports (ITARs) are critical for the program to be able to make decisions on progress and advancement. Ideally, the ITAR process consists of a brief face-to-face discussion with the resident near the start of the rotation to discuss learning goals, resident progress discussed at the middle of the rotation, and at the end of a rotation, followed by completion of the End of Rotation ITAR. We advise residents to prompt you to set up a time to meet in advance of the rotation. The ITAR is part of the resident's permanent record in the program.
- ITAR forms are distributed on One45 by site administrative staff to the main preceptor/head evaluator identified for a particular rotation. If there are changes to this role, please let your site administrator know. ITAR forms are distributed 1-2 weeks prior to the end of a rotation for completion and should ideally be completed **within 2 weeks after the end of a rotation.**

- A mid-point ITAR is required for:
 - Horizontal/longitudinal rotations (including “call-back” days for Family Medicine), every 3-4 months
 - Block rotations greater than 4 weeks at the mid-point of the block.
 - At any time, if a resident is not progressing as expected (these can be requested from the site administrator).
- ITARs should provide a narrative assessment, ideally with examples, on a resident's performance, with the **goal of providing competency committee members** (who may not work directly with the resident otherwise) **information on whether the resident is meeting competencies and how**. The **benchmark document** (https://entrada.med.ubc.ca/community/familypracticepostgr:family_practice_postgrad_program_section=view-file&id=39211&download=latest) can provide guidance in framing these narratives. Further, they should be informed by:
 - Multiple observers via Field Notes, end-of-shift forms, and similar;
 - In-person (direct) observation of resident performance;
 - Discussions with colleagues, including allied health professionals;
 - Review of cases, and;
 - Review of progress notes and other clinical documentation.

ITAR Appeals

The resident can appeal ITARs based on substantive issues of the process that affect the fairness of the assessment.

The most common reason for the appeal of an ITAR:

- Not clearly hearing that you had concerns about their performance during the rotation.
- Significant delay in filling in an ITAR

This is why a written Midpoint ITAR is required if performance concerns are noted.

Residents need **at least 25% of the time left in rotation** to demonstrate they are working to resolve any performance concerns.

Should an appeal of an ITAR you completed be successful, your concerns as noted in the narrative, are **not** ignored. The responsible Competency Committee must still support the resident to mitigate your concerns.

The ITAR appeal policy can be found here:

UBC Family Practice Residency Program in Training Assessment Report (ITAR) Appeal Policy

1. Appeal Process

1.1 The resident may appeal an ITAR when the reporting preceptor has stated that the resident does not meet expectations overall at the end of a rotation.

1.2 The resident must communicate an intention to appeal to the program director, or designate, in writing, within fourteen (14) calendar days of being notified by One45 (or equivalent) of an ITAR which states there was a failure to meet expectations overall in the rotation.

1.2.1 Should the resident enter a leave (medical/ personal/ vacation) within that fourteen (14) calendar day period, the time limitation of the appeal will be paused from the time of the entered leave on RMS until the time of the entered return on RMS. Time from the submission of the ITAR until the beginning of an entered leave shall be counted as part of those fourteen (14) calendar days.

1.3 The program director or designate will review the appeal and will determine whether the request for appeal is based on one of the grounds of appeal set out in this policy. If the request for appeal is not based on a ground of appeal set out in this policy the program director or designate may dismiss the appeal without further review.

1.4 The program director or designate can grant the appeal if conditions for appeal are clearly met.

1.5 If the program director or designate is unable to determine whether conditions for appeal have been met, they will forward the appeal to the Program Competency Committee for review and decision.

1.6 If the original ITAR decision is reversed, either by the program director or designate, or the Program Competency Committee, the program director designate will amend the ITAR on one45 (or equivalent platform) in order to show the successful result of the appeal, with an explanation of why the appeal was successful. The original narrative of the ITAR as originally provided by the preceptor will remain unaltered.

1.7 If the appeal is denied, and the original ITAR decision is upheld, either by the program director or designate or Program Competency Committee, the resident will be informed in writing of this decision by the program director or designate.

2. Grounds for Appeal

2.1 The reasons for failing to meet expectations are not pertinent to performance of the resident within the time frame of the actual clinical rotation.

2.2 The assessment information used to inform the ITAR was not from the preceptors or Allied Health Professional associated with the rotation.

2.3 The resident was not made aware of the performance concerns prior to the end of the rotation. This would include direct communication either in-person or virtually, followed by some sort of written confirmation (email, midterm ITAR, or equivalent, or competency committee minutes).

2.4. For ITARs related to clinical experiences, the resident was not provided adequate time to address the performance issues. Adequate time is a minimum of 25% remaining in the clinical experience. If the program director or designate is unable to determine whether there was adequacy of time provided the decision will lie with the Program Competency Committee (see item 1.5)

2.4.1 The duration of the clinical experience is determined by the dates entered in One45.

2.4.2 Should the resident have vacation, medical and/or educational leave scheduled within the rotation, the minimum 25% is determined by the scheduled dates, not the dates the resident was present on the rotation.

2.4.3 Should the resident have vacation and/or educational leave scheduled after a unfavorable mid-point ITAR, the resident will have the opportunity to reschedule their vacation and/or educational leave, to remain on the learning experience and meet the competencies required for successful completion.

2.4.3.1 The initiation of this deferment of vacation/educational leave must be done by the resident immediately following (within 48 hours) the mid-point ITAR, to ensure the adequate 25% clinical exposure remains available.

2.5 The End Rotation ITAR documenting “Resident Not Meeting Expectations” was submitted greater than fourteen (14) calendar days from completion of the rotation in question

The items 2.1, 2.2, 2.3 2.4 and 2.5 are grounds for appeal and the presence of these do not guarantee success of the appeal.

3. Consequences of Appeal

- 3.1.** If an appeal is successful, the evaluative comments on the ITAR remain and an addendum is attached explaining the reasons for the successful appeal and reversal of the “not meeting expectations” decision.
- 3.2.** If an appeal is successful or unsuccessful, further steps may be directed by the site-level competency committee (SLCC) and/or program competency committee (PCC).

Reviewed and approved by the PGEC on October 3, 2025